جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : حنان احمد على احمد
المحافظة : البحر الاحمر **الحي / المدينة :** الغردقه **العنوان :** .
رقم البطاقة : 27308300103843 **رقم التليفون :** 01007276868 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 30-08-1973 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: حنان احمد على احمد
National ID: 27308300103843

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Zoledronic acid for bone protection during hormonal therapy**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية



حنان
احمد على احمد
مساكن جمعية الاسكان والتعمير
ثان الغردقة - البحر الاحمر

٢ ٧٣ ٠٨ ٣٠ ٠١ ٠٣٨ ٤٣

LB0383011

٢٧٣٠٨٣٠٠١٠٣٨٤٣ ٢٠٢٥/٠٣

معهد فنى تجارى للحاسب الآلى

انثى مسلمة متزوجة

احمد عطا زرزور على

١٤ البطاقة سارية حتى ٢٠٣٢/٠٣/٠٨

جوستافروسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الست المصرية هي صحة مصر

MDT FORM 28/7/2025 G

Name	[illegible]	MRN	179150
Age	52	ID	29208210201252
Menopausal status	ombole	Phone	01007776264
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

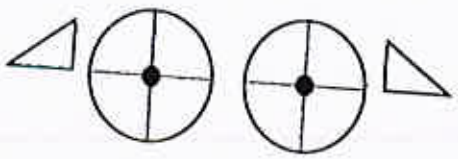
BX: IDC grade II

ER:	PR:	HER 2:	Ki67:
7/8	2/8	3+	25%

Sonomammography:

Mets Workup: end of NEADJTCHP cycle 4
last cycle 24/4/2025
Internal Surg. with Rad. Onc.
[illegible]

R L



TNM
ypT0 N1/9

Recommendations:
2 Appeal of Dr. Kadiyala + AI + Zoladex
5th endocrine PORT

MDT OUTCOME & PLAN for - AI Kadiyala
- Zoladex + AI

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: حنان احمد علي احمد	
MR. : 5912	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient was diagnosed with left breast cancer, clinical stage T3 N1 M0, HER2-positive, hormone-receptor negative. Neoadjuvant therapy with TCHP was administered for six cycles (TCHPX6). Following breast-conserving surgery (BCS) with negative margins, adjuvant therapy was initiated with KATCYLA, FEMARA, and ZOLDEX; the last KATCYLA dose was given in December 2025 (C6).

For cycle 8 (C8), the patient received KADCYLA 260 mg combined with FEMARA, with ZOLADEX scheduled for cycles 9 and 10. Pharmacy records confirm the patient's weight as 83 kg at the time of the C8 administration.

During cycle 9 (C9), the patient was administered Kadcyla 260 mg plus FEMARA and ZOMETA. Laboratory results on the day of treatment showed creatinine 0.99 mg/dL, calcium 8.9 mg/dL, and albumin 1.15 g/dL.

In cycle 10 (C10), the patient received Kadcyla together with FEMARA. An echocardiogram performed on 12 April 2026 demonstrated a left ventricular ejection fraction of 62 %.

On 31 May 2026, the patient's disease was restaged as left breast cancer cT2 N1 M0, invasive ductal carcinoma, grade 2, estrogen-receptor 7/8 positive, progesterone-receptor 7/8 positive, HER2 23 % positive, Ki-67 25 %. The treatment plan included continuation of TCHP, followed by Kadcyla, ZOLADEX, and FEMARA. The patient was scheduled for cycle 11 (C11) with Kadcyla 260 mg, ZOLADEX, and FEMARA, with an echocardiogram showing an ejection fraction of 62 % on 12 April 2026.

For cycle 12 (C12), Kadcyla and FEMARA were administered, and laboratory work performed on 21 June 2026 was accepted as satisfactory.

A PET scan performed on 22 August 2026 revealed low-grade metabolic activity associated with left breast parenchymal thickening and newly developed low-grade metabolically active left breast skin thickening. Additional low-grade FDG uptake was noted in several vertebrae, the pelvis, and the sacrum, with a maximum SUV of 2.8. The plan is to complete two years of ZOLADEX therapy and then continue FEMARA with close follow-up.

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : محمد رضا أبو اليزيد كيلاني

يعتمد ,

Name : hanan ahmed ali
Branch : Portable
Ref.Physician Prof :

Date : 19-01-2026 11:11
Medical No. : GRI-27308300103843
Ins.Company :

MIULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- Known case of left breast cancer showed diffuse skin thickening of the left breast with underlying heterogenous densitys
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

❖ **No hepatic or pulmonary metastatic lesions.**

Dr. Asser Ahmed

Dr. Asser Ahmed

Name : hanan ahmed ali
Branch : Portable
Ref.Physician Prof :

Date : 19-01-2026 11:11
Medical No. : GRI-27308300103843
Ins.Company :

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- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

❖ **No hepatic or pulmonary metastatic lesions.**

Dr. Asser Ahmed

Dr. Asser Ahmed

National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

GUSTAVE ROUSSY

Patient Name	: الأستاذه / حنان احمد على احمد	Request No.	: 13002344
Patient No.	: 27308300103843	Request Date	: 22/06/2025
Path No.	: SR-855/25	Verification Date	: 29/06/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 51 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical data
Left breast cancer

Nature of specimen
Block &slide revision

Gross specimen
One block &12 slides were received

Microscopic examination
-Breast tissue with no residual invasive tumor
-Lymph nodes positive (1/9) with extracapsular extension

Diagnosis
Left breast mass , slide reviosion of BCS slides, post NACT :
-Breast tissue with no residual invasive tumor
-Lymph nodes positive (1/9) with extracapsular extension
Comment : No paraffin blocks for lymph nodes were provided by pateint to do IHC

Signature
Dr. Walaa Ibrahim Ghoneim
Dr.Heba Hassan Adly

Verified by :

CAP

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY

Patient Name	: الأستاذة / حنان احمد على احمد	Request No.	: 13006816
Patient No.	: 27308300103843	Request Date	: 21/07/2025
Path No.	: SR-992/25	Verification Date	: 21/07/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 51 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

IPT:

* ER: Positive (score 7/8).
* PR: Positive (score 7/8).
* HER2: Positive (score 3+).
* Ki67: Positive in 25% of tumor cells.

Signature
Dr.Walaa Ibrahim Ghoneim
Dr.Heba Hassan Adly

Verified by :

CAP
غرفة رقم (4) – الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Cornicle - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY

Patient Name	: الأستاذه / حنان احمد على احمد	Request No.	: 13006816
Patient No.	: 27308300103843	Request Date	: 21/07/2025
Path No.	: SR-992/25	Verification Date	: 21/07/2025
Sex	: Female	Financial Category	: جوستاف روسي - فئة الدولة
Age	: 51 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data
Left breast mass

Nature of specimen
Slide revision of tru cut biopsy

Gross examination
5 slides labelled by (675434) were received

Microscopic examination
* Histological type: Invasive duct carcinoma
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade 2
* DCIS: Positive, low grade and intermediate grade solid , cribriform , comedo , 5 %
* Angioinvasion: Positive
*Microcalcification : Positive
*Perineural invasion: Absent

• IPT revision:
* ER: Moderate nuclear reaction in 80 % tumor cells (Allred score 7/8)
* PR: Strong nuclear reaction in 50 %tumor cells (Allred score 7/8)
* HER2: Strong complete membrane reaction in >10 % of tumor cells (score 3+)
* Ki 67: 25 % nuclear reaction of tumor cells

Diagnosis
Left breast mass, Slide revision of tru cut biopsy :
Invasive duct carcinoma, Grade 2, with associated ductal carcinoma insitu, low &intermediate grade (solid, cribriform, comedo types), 5 %

Verified by :

CAP
غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Nile Hospital
Hurghada
مستشفى النيل

Patient Name	حنان احمد علي		
Patient MRN	N20225334	Age	45 Y Gender :Female
Patient Type	Outpatient	Request Time	30/10/2024 20:12 PM
Room Number	كشف عيادة خارجية	Reporting Time	30/10/2024 20:50 PM
Ref. Doctor	Mostafa Mohammed Ali Hasanen	Order Number	1743709

Bilateral breast sonomammography revealed:

Patient complaining of right breast lump

- Breast parenchyma composition B
- Mixed fatty glandular tissue of both breasts.
- Left breast shows a well defined irregular high density lesion, seen at the upper outer quadrant posterior third with fine speculation.
- Bilateral scattered foci of round calcifications.
- Normal skin thickness& normal nipple areola complex.

Complementary U/S revealed:

- Normal thickness and echogenecity of the fibroglandular layer of both breasts
- **Left breast shows a well defined irregular shaped hypoechoic heterogeneous breast mass with indistinct margin, with vertical orientation, seen at 3 o'clock direction, measuring (20x14 mm), mass shows posterior shadowing, surrounded with thin halo of edema, with color Doppler it has no detectable vascularity**
- **Evidence of enlarged left axillary lymph nodes with rounded shape, showing eccentric hilum other with lost hilum ... suspicious**
- No dilated major retro-areolar ducts.
- Normal skin thickness and nipple areola complex.
- Normal axillary vessels.

OPINION:

- **Left breast mass, (BI-RADS V)** ..... *(for Histopathological correlation).*

Best regards
Dr Mostafa Hassanin

Nile Hospital
Hurghada
مستشفى النيل

Treating Doctor	محمد يسرى محمد احمد مصطفى حجاب	Patient ID	N20225334
Attendance Date	10/10/2026 12:41:22 PM	Patient Name	حنان احمد على احمد
Request No	1206076	Confirm Date	10/10/2026 14:17

Chemistry Unit

Test Names	Result	Unit	Reference Range
Kidney Function Test			
Blood Urea	30	mg/dl	[10 - 50]
Blood Urea Nitrogen(BUN)	14.02	mg/dl	[5.00 - 23.00]
Serum Creatinine	0.79	mg/dl	[0.50 - 1.40]
Liver Function Test			
AST (SGOT)	21	U/L	[0 - 42]
ALT (SGPT)	17	U/L	[0 - 40]
Serum Electrolytes Report			
Serum Electrolytes Report			
Ionized Calcium(Ca++)	1.27	mMol/L	[1.10 - 1.30]
Serum Calcium (Total)	9.6	mg/dl	[8.6 - 10.0]

Lab Director

Nile Hospital
LAB
01000094593

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فاكس : ٣٥٥٠٩٧٧ (٠٦٥) (٢+)
ص.ب. ٦٤٩ الغردقة

Lab Test
[[DOCTYPE:LAB_TEST]]

Nile Hospital
Hurghada
مستشفى النيل

Treating Doctor	محمد يسرى محمد احمد مصطفى حجاب	Patient ID	N20225334
Attendance Date	10/10/2026 12:41:22 PM	Patient Name	حنان احمد على احمد
Request No	1206076	Confirm Date	10/10/2026 16:13

Hematology Unit

Test Names	Result	Unit	Reference Range	
Hematology Report				
Haemoglobin	11.9	gm/dl	[12.0 - 15.0]	↓
Haematocrit	38.8	%	[36.0 - 46.0]	
Red cell count	5.19	mil/cmm	[3.80 - 4.80]	↑
Red Cell indices				
MCV	74.8	fL	[83.0 - 101.0]	↓
MCH	22.9	Pg	[27.0 - 32.0]	↓
MCHC	30.7	gm/dl	[32.0 - 36.0]	↓
RDW	17.0	%	[11.6 - 14.0]	↑
Total Leucocytic Count				
WBC	6400	/cmm	[4000 - 11000]	
Differential Count				
Basophils	0	%	[0 - 1]	
Eosinophils	2	%	[1 - 6]	
Staff	1	%	[0 - 6]	
Segmented	54	%	[35 - 75]	
Lymphocytes	33	%	[20.0 - 45.0]	
Monocytes	10	%	[2 - 8]	↑
NEUT absolute count	3.6		[2.0 - 7.0]	
Lymphocytes absolute count	2.0		[1.0 - 3.5]	
Monocytes absolute count	0.6		[.2 - 1]	
Eosinophils absolute count	0.1		[.02 - .5]	
Platelets Count	228000	/cmm	[150000 - 450000]	

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فاكس : ٣٥٥٠٩٧٧ (٠٦٥) (+٢)
ص.ب. ٦٤٩ الغردقة

Lab Test
[[DOCTYPE:LAB_TEST]]

Nile Hospital
Hurghada
مستشفى النيل

Echocardiography Report

Name of the patient	Hanan Ahmed Aly	Age	53
Referring Doctor		Sex	female
Operator	Dr. Mohamed Yousry	Date	10-10-2026

2D Dimensions

RV		1.5-2.5 cm	Ao Root	3.6	1.5-4.0 cm
IVSD	1.0	0.7-1.1 cm	LAD	4.0	1.5- 3.5 cm
LVEDD	5.2	2.5-5.6 cm	LV EF%	57 %	56- 76%
LVESD	3.6		FS%	35 %	25-40%
PWT	1.0	0.7-1.1 cm			

Comments :

- Normal LV internal dimensions with good systolic functions , EF by m-mode 57 %.
- No RWMA at rest .
- Normal LA dimension.
- Diastolic dysfunction grade I .
- Degenerative changes of mitral valve leaflets with mild regurge .
- Trileaflet aortic valve with trivial regurge .
- mild tricuspid regurge with PASP 28 mmhg.
- Intact IAS & IVS by TTE.
- No PDA or Coarctation
- No intra-cavitary masses or thrombi by TTE.
- No pericardial effusion.

Conclusion

- Diastolic dysfunction grade I, mild mitral regurge.

Dr .Mohamed Yousry
Cardiology consultant .

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فاكس : ٣٥٥٠٩٧٧ (٠٦٥) (٢+)
ص.ب. ٦٤٩ الغردقة

Lab Test
[[DOCTYPE:LAB_TEST]]

Nile Hospital
Hurghada
مستشفى النيل

Patient Name	حنان احمد على احمد		
Patient MRN	N20225334	Age	47 Y Gender :Female
Patient Type	Outpatient	Request Time	10/10/2026 12:41 PM
Room Number	عيادة القلب	Reporting Time	10/10/2026 13:16 PM
Ref. Doctor	محمود Ysra Mhmd Ahmd	Order Number	1802070

Bilateral breast sonography revealed:

History of left lumpectomy, underwent chemo and radiotherapy

Left breast:

- No detectable residual nor recurrence
- Relatively diffuse skin thickening …accepted post-radio
- Normal thickness and echogenicity of the fibroglandular layer of left breast
- No dilated major retro-areolar ducts.
- No enlarged axillary LNs.
- Normal axillary vessels.

Right breast:

- Normal thickness and echogenicity of the fibroglandular layer of left breast with no solid or cystic masses could be detected.
- No dilated major retro-areolar ducts.
- Normal skin thickness and nipple areola complex.
- Few enlarged axillary LNs, up to 12 mm with preserved shape and hilum … inflammatory.
- Normal axillary vessels.

OPINION:

- Unremarkable study

Best regards
Dr Mostafa Hassanin

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فاكس : ٣٥٥٠٩٧٧ (٠٦٥) (٢+)
ص.ب. ٦٤٩ الغردقة